

Competitive Pricing and Supplier Landscape for the Top Five Therapeutic Peptide Categories — 19 August 2026

Prepared for: George stakeholders, South Africa

Author: Manus AI

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Regulatory notice. This report addresses regulated finished-dose procurement and strategic supplier positioning, not medical treatment or dosing. Product-specific **SAHPRA registration, approved professional information, current certificate status, licensed wholesaler/supplier status and physical availability must be verified before any purchase, tender, distribution, switching or investment decision.** Nothing in this report supports grey-market sourcing, compounding, research-use supply, or any route around South African medicine registration.

Executive summary

The five categories divide into three procurement archetypes. **Tirzepatide and South African dulaglutide are presently single-originator finished-dose markets:** Eli Lilly controls regulated Mounjaro supply and, outside China, Trulicity remains Lilly-controlled in the reviewed markets. ¹ ¹⁶ **Semaglutide and insulin analogues offer the clearest immediate regulated competition in South Africa:** the 5 August 2026 National Department of Health (NDoH) price database lists Sun/Ranbaxy's Semagard alongside Novo Nordisk's Ozempic and Extensior, while U-100 insulin glargine has multiple listed suppliers and materially different Single Exit Prices (SEPs). ⁵ ⁶ **Octreotide/lanreotide is a split market:** short-acting octreotide is a mature sterile-generic segment, whereas octreotide microsphere depots and lanreotide gel prefilled syringes remain complex, concentrated and service-intensive. ²² ²⁴

The strongest measurable South African competitive price signal is semaglutide for type 2 diabetes. At matched 1.5 mL concentration/volume, the official SEP is **ZAR 1,437.78 for Ozempic, ZAR 1,222.11 for Novo's Extensior and ZAR 1,161.00735 for Sun/Ranbaxy's Semagard**; the latter is 19.25% below Ozempic and 5.00% below Extensior. ⁵ ⁶ Glargine also shows a wide nominal range: **ZAR 981.51 for Lantus, ZAR 775.90 for Basaglar and ZAR 582.90 for Endulin Select**, each recorded as 1,500-unit U-100 packs, although exact device/form and live availability must be reconciled before a like-for-like award. ⁵ ¹²

These are regulated private-market SEPs, not public tender prices or manufacturer net receipts. ⁵

Tirzepatide offers little immediate supplier leverage but a useful presentation benchmark. South Africa's official 2.5 mg Mounjaro prices imply **ZAR 3,559.25 for four single-dose vials** versus **ZAR 3,605.59 for one four-dose KwikPen**, before permitted dispensing and ancillary device costs. ¹ This small measured acquisition-price gap should be analysed through total delivered cost, including needles or syringes, training, wastage, cold-chain handling and continuity by strength. Sandoz disclosed that two proposed generic tirzepatide autoinjector ANDAs were accepted for FDA review in June 2026, but no approval or South African filing was verified by the decision date. ³

Dulaglutide is strategically different from synthetic incretin peptides. It is a CHO-derived glycosylated GLP-1-IgG4 Fc fusion biologic, and Lilly's worldwide Trulicity revenue fell from US\$7.132 billion in 2023 to US\$4.276 billion in 2025, while Boan Biotech's Boyouping became the first approved dulaglutide biosimilar in China in August 2025. ¹⁵ ¹⁶ South Africa nevertheless has one publicly verified Trulicity presentation, at **ZAR 1,223.83 SEP for four pens**, and no verified local follow-on or public tender price. ⁵ The category is therefore a medium-term licensing/biosimilar watch opportunity, not a near-term API procurement play.

Somatostatin depots carry the largest visible South African pack costs in the portfolio: **ZAR 21,600.79 SEP for the listed Sandostatin LAR 20 mg pack** and **ZAR 28,670.37 SEP for one Somatuline Autogel 120 mg prefilled syringe**, based on 26 May 2026 registry observations. ²⁵ ²⁶ The Sandostatin LAR registry pack descriptor is internally ambiguous and must be confirmed with the supplier before comparison. ²⁵ UK evidence shows measurable follow-on list discounts—10.0% for Teva Olatuton 20 mg versus Sandostatin LAR and 20.0% for ADVANZ lanreotide 120 mg versus Ipsen—but reimbursement tariffs remain at originator levels, illustrating why list-price savings do not automatically translate into payer savings. ²⁷ ²⁸

George's recommended portfolio stance is therefore: **compete now** in same-presentation semaglutide diabetes products and glargine U-100; **negotiate continuity and services rather than promise a second source** for tirzepatide and dulaglutide; **run separate molecule/formulation lots** for insulin and somatostatin analogues; and **develop regulated partnerships only through a named MAH, approved sites and SAHPRA change control**. Across all five categories, the purchase specification should cover the entire regulated system—drug product, device, ancillary consumables, cold chain, training, pharmacovigilance, recall execution and supply assurance—not nominal peptide API alone.

¹⁰ ²⁰ ²³

1. Scope and comparability rules

This report compares regulated finished-dose tirzepatide, semaglutide, representative basal and prandial insulin analogues, dulaglutide, and short-acting/depot octreotide and lanreotide. It excludes compounded, research-use, falsified, grey-market and unverified products. SAHPRA has specifically warned that compounded or unregistered GLP-1 and GIP/GLP-1 products are unlawful and may present sterility, identity and dose risks. [4](#)

The analysis uses the following comparability rules.

Rule	Application
Price bases remain separate	South African SEP, UK NHS indicative/Drug Tariff price, US WAC/list price, US manufacturer cash offers, Australian DPMQ/PBS reimbursement and public-tender prices are reported as different constructs. None is treated as another market's net procurement price. 1 5 17 18 27 29
No currency conversion	Prices remain in source currency because channel, pack, indication, tax, rebate and device differences dominate a simple FX conversion. The report does not create an international price league table.
Normalize only inside a defensible class	Per-dose, per-100-unit or four-week arithmetic is used only when the source pack and deliverable units permit it. Oral and injectable semaglutide are not compared per milligram; octreotide IR, octreotide LAR and lanreotide gel are not pooled per milligram. 7 23
Indications remain separate	Diabetes and weight-management brands/presentations are not treated as automatic substitutes. In particular, South African Ozempic/Extensior/Semagard diabetes pens are not assumed to substitute for Wegovy weight management. 6 7
Device and formulation are part of the product	Vials, disposable autoinjectors, multidose pens, cartridges, concentrated pens, microsphere kits and gel prefilled syringes have distinct consumables, training, wastage and human-factors costs. 8 10 14 23
Registration evidence is not stock evidence	NDoH notes that discontinued medicines may remain in its price database; a price/registration row therefore does not

	establish physical stock or shortage-free supply. 30
Tender evidence requires an award	SEP or a bid specification is not called a tender price. No current public award price was verified for tirzepatide, semaglutide, dulaglutide, analogue insulin, or depot octreotide/lanreotide in the sources reviewed. 1 5 13 31

2. Cross-category measured price benchmarks

Table interpretation. Every row below is a measured public price fact on its stated basis. Rows should be compared only within the same country, molecule, concentration, presentation and channel where the table explicitly identifies a like-for-like relationship.

Category	Country and product	Exact public price	Basis and preserved date	Defensible interpretation
Tirzepatide	South Africa, Mounjaro 2.5 mg single-dose vial, one vial	ZAR 889.81 ; four-vial arithmetic ZAR 3,559.25	Private-sector SEP, VAT-inclusive, effective 3 Feb 2026; NDoH database dated 5 Aug 2026. 1	One vial is one weekly unit; vials are not equivalent to multidose pens
Tirzepatide	South Africa, Mounjaro 2.5 mg four-dose KwikPen	ZAR 3,605.59 ; ZAR 901.40/dose	Private-sector SEP, VAT-inclusive, effective 3 Feb 2026; database 5 Aug 2026. 1	Needles and dispensing are additional; this is not a tender
Tirzepatide	South Africa, Mounjaro 12.5 or 15 mg four-dose KwikPen	ZAR 8,167.60 ; ZAR 2,041.90/dose	Private-sector SEP, effective 3 Feb 2026; database 5 Aug 2026. 1	Equal pack sizes across strengths does not imply equal price per milligram.
Tirzepatide	UK, Mounjaro 2.5 mg four-dose KwikPen	GBP 133.00 ; GBP 33.25/dose	NHS Drug Tariff Part VIIIA Category C, Aug 2026. 2	Reimbursement price, no cash or Lilly receipt.
Tirzepatide	US, Mounjaro four single-dose pens	USD 1,112.16/28 days ; USD 278.04/dose	Manufacturer WAC/list, accessed 19 Aug 2026. 32	Not patient net realised after rebate

Tirzepatide	US, Zepbound 2.5 mg four vials or one KwikPen	USD 299/28 days; USD 74.75/dose	LillyDirect regular self-pay, 19 Aug 2026. 33	Manufacturing channel; no or payer net
Semaglutide	South Africa, Ozempic 1.34 mg/mL, 1.5 mL pen	ZAR 1,437.78	Private-sector SEP effective 3 Feb 2026; database 5 Aug 2026. 5	Same-volun benchmark Extensior ar Semagard; i tender price
Semaglutide	South Africa, Extensior 1.34 mg/mL, 1.5 mL pen	ZAR 1,222.11	Private-sector SEP effective 20 Jul 2026; database 5 Aug 2026. 5	15.00% belc matched Oz SEP; still a N product.
Semaglutide	South Africa, Semagard 2 mg/1.5 mL pen	ZAR 1,161.00735	Private-sector SEP effective 28 Jul 2026; database 5 Aug 2026. 5 6	19.25% belc matched Oz and 5.00% b Extensior; approved fo diabetes, nc Wegovy sub
Semaglutide	South Africa, Wegovy injection 2.4 mg, four-dose pen	ZAR 3,745.64; arithmetic ZAR 936.41/dose	Private-sector SEP effective 3 Feb 2026; database 5 Aug 2026. 5	Weight-managemer product; do compare to Ozempic di packs.
Insulin analogue	South Africa, Lantus glargine U-100, 3 mL × 5	ZAR 981.51/pack; ZAR 65.43/100 units	Private-sector SEP effective 23 Jan 2026; database 5 Aug 2026. 5	Exact device must be cor before like-l tendering.
Insulin analogue	South Africa, Basaglar glargine U-100, 3 mL × 5	ZAR 775.90/pack; ZAR 51.73/100 units	Private-sector SEP effective 3 Feb 2026; database 5 Aug 2026. 5	Nominally matched concentrati me; device c still matters
Insulin analogue	South Africa, Endulin Select glargine U-100, 3 mL × 5	ZAR 582.90/pack; ZAR 38.86/100 units	Private-sector SEP effective 9 Mar 2026; database 5 Aug 2026. 5	Lowest liste nominal gla benchmark; does not ma exact device explicit.
Insulin analogue	South Africa, Tresiba degludec U-	ZAR 1,117.85/pack;	Private-sector SEP effective 3 Feb	Different ba molecule ar

	100, 3 mL × 5	ZAR 74.52/100 units	2026; database 5 Aug 2026. ⁵	profile; do n with glargin
Insulin analogue	UK, Lantus versus Semglee glargine U-100, five pens	GBP 34.75 vs GBP 29.99	NHS indicative prices, 19 Aug 2026. ¹²	Semglee is : below Lantu broadly mat pen-pack lis confidential prices rema unknown.
Dulaglutide	South Africa, Trulicity 1.5 mg/0.5 mL, four pens	ZAR 1,223.83; ZAR 305.96/dose	Private-sector SEP effective 24 Feb 2026; database 5 Aug 2026. ⁵	Originator p record; no l follow-on or price verifie
Dulaglutide	UK, Trulicity four pens, all listed strengths	GBP 73.25; GBP 18.3125/dose	NHS indicative/Drug Tariff, checked 19 Aug 2026. ¹⁷	Same pack across stren not a per-m or manufact net.
Dulaglutide	US, Trulicity four pens	USD 1,006.93/month; USD 251.73/dose	WAC accessed 1 Jan 2026; page checked 19 Aug 2026. ¹⁸	Wholesaler patient cost realised pric
Somatostatin analogue	South Africa, Sandostatin IR 0.05 mg/1 mL, five ampoules	ZAR 1,600.20/pack; ZAR 320.04/ampoule	Private-sector SEP collected 26 May 2026; accessed 19 Aug 2026. ²⁶	Short-acting presentatio comparable monthly de
Somatostatin analogue	South Africa, Sandostatin LAR 20 mg, listed pack	ZAR 21,600.79	Private-sector SEP collected 26 May 2026; accessed 19 Aug 2026. ²⁵	Exact saleak composition be confirme because the registry des is ambiguou
Somatostatin analogue	South Africa, Somatuline Autogel 120 mg PFS	ZAR 28,670.37/syringe	Private-sector SEP collected 26 May 2026; accessed 19 Aug 2026. ²⁵	One prolong release PFS; cross-molec mg compari
Somatostatin analogue	UK, Olatuton versus Sandostatin LAR 20 mg	GBP 719.40 vs GBP 799.33	NHS indicative prices, accessed 19 Aug 2026. ²⁷	Olatuton is . lower at list show GBP 7 Drug Tariff reimbursen

Somatostatin analogue	UK, ADVANZ versus Ipsen lanreotide 120 mg	GBP 749.60 vs GBP 937.00	NHS indicative prices, accessed 19 Aug 2026. 28	ADVANZ is 2 lower at list show GBP 9 Drug Tariff reimbursed
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3. Supplier landscape

Category	Regulated supplier/control point	Verified role	South African competitive implication
Tirzepatide	Eli Lilly and Company / Eli Lilly (SA) (Pty) Ltd	Originator, patent owner, finished-dose controller and local price applicant; Lilly markets six strengths in jurisdiction-specific vials/pens. 1 8	Only bankable regulated upstream owner; negotiate allocations, strength continuity and services.
Tirzepatide	Aspen Pharmacare	Lilly-appointed South African/sub-Saharan distribution and promotion partner; no reviewed primary source identifies Aspen as independent tirzepatide API or finished-dose manufacturer. 34	Local route-to-market, not a second source.
Tirzepatide	Sandoz Group AG	Two proposed generic autoinjector ANDAs accepted for FDA review in June 2026; no approval or SA filing verified. 3	Monitor, but exclude from 2026 base-case supply.
Semaglutide	Novo Nordisk A/S / Novo Nordisk (Pty) Ltd	Originator, API/formulation owner and finished-dose supplier for Ozempic, Wegovy and Extensor. 5 35	Incumbent with portfolio leverage; Extensor is a value brand, not independent dual sourcing.

Semaglutide	Sun Pharma / Ranbaxy Pharmaceuticals (Pty) Ltd	SAHPRA-approved Semagard generic finished-dose pens for type 2 diabetes. 5 6	First verified independent regulated South African competitor and strongest current negotiation lever.
Insulin analogues	Sanofi	Lantus/Toujeo originator; lispro and aspart biosimilar/follow-on supplier in regulated markets. 36 37	Broad cross-molecule counterparty and glargine incumbent.
Insulin analogues	Eli Lilly	Humalog originator and Basaglar/Abasaglar glargine competitor. 5 38	Strong local applicant/device base; credible U-100 glargine competition.
Insulin analogues	Novo Nordisk	NovoRapid/Fiasp aspart and Tresiba degludec originator/manufacturer. 39	Concentrated degludec position; significant prandial installed base.
Insulin analogues	Biocon Biologics / Viatris-Mylan	Biocon has integrated insulin drug-substance, vial, cartridge and device capability; Mylan is the South African applicant for Endulin Select in the price database. 5 14	Priority diligence target for glargine competition; verify current rights, sites and live supply.
Dulaglutide	Eli Lilly / Lilly Kinsale	Originator and global finished-dose controller; EMA identifies Kinsale as a dulaglutide drug-substance site. 19	Single-source exposure in South Africa.
Dulaglutide	Shandong Boan Biotechnology	Developer/manufacturer of Boyouping, approved in China in August 2025 as the first dulaglutide biosimilar. 15	Technical proof of follow-on feasibility; no South African authority without SAHPRA approval and territorial rights.

Octreotide	Novartis	Sandostatin IR and LAR originator/MAH. 22 40	Incumbent; use registered generics/follow-ons and service terms as leverage.
Octreotide	Teva / Pharmathen	Teva markets the first US generic Sandostatin LAR and UK Olatuton; FDA-recognised labelling names Pharmathen as manufacturer. 23 41	Most relevant complex-depot challenger; local registration, pack and supply must be confirmed.
Lanreotide	Ipsen / Acino South Africa	Ipsen originator; SAHPRA's 2025 recall identifies Acino Pharma (Pty) Ltd in the South African supply chain. 24 42	Existing channel but device-remediation and recall-closure diligence are essential.
Lanreotide	Cipla/InvaGen and ADVANZ PHARMA	Regulated US and European follow-on suppliers respectively. 43 44	Potential strategic alternatives, but no South African supply is assumed without local registration.
Upstream peptide capability	Bachem; CordenPharma	Bachem documents GMP octreotide acetate with DMF/CEP; CordenPharma documents peptide API and sterile injectable capability. 45 46	Capability evidence only; neither API/CDMO route bypasses finished-dose registration.

4. Detailed category analysis

4.1 Tirzepatide: high growth, originator control and strength-linked continuity

Measured facts

The 5 August 2026 NDoH database lists South African Mounjaro single-dose vials at 2.5, 5, 7.5 and 10 mg and four-dose KwikPens at all six strengths from 2.5 to 15 mg, with Eli Lilly

(SA) (Pty) Ltd as applicant and the products classified as originator. ¹ Aspen states that SAHPRA approved the KwikPen and chronic weight-management indication; no South African Zepbound record was found in the official price evidence reviewed. ³⁴

Lilly estimates compound-patent expiry in 2036 in the US, 2037 in major European countries and 2040 in Japan, while noting that formulation, process, device and use protections may extend further. ⁸ FDA's July 2026 generic-peptide guidance highlights manufacturing route, impurity, higher-order structure, biological activity and innate immune response considerations for proposed generic peptide products. ⁴⁷ Tirzepatide's US shortage was declared resolved by FDA in December 2024, although FDA warned that intermittent local disruptions could continue; Lilly's supply page currently reports pen and vial availability. ⁴⁸ ⁴⁹

Analysis and procurement implication

Analysis. The immediate negotiating objective is not an unrealistically low commodity-peptide price. It is a bankable allocation and service package from Lilly/Aspen. George should contract by exact NAPPI, registration, indication, presentation and strength, because titration creates linked demand across strengths and a missing intermediate presentation can disrupt continuity. The measured 2.5 mg vial-versus-KwikPen difference is only **ZAR 46.34 per four weeks** before ancillary costs, calculated from the official SEPs; total-cost evaluation should therefore include syringes or pen needles, training, dispensing, wastage and return logistics. ¹

A 2026 business case should assign no launch probability to unapproved Sandoz supply. A strategic option value may be retained through regulatory monitoring, but any future South African inclusion should follow primary evidence of SAHPRA filing/approval, patent freedom, approved sites and commercial availability. ³ ⁸

4.2 Semaglutide: South Africa's clearest new competitive peptide market

Measured facts

The official 5 August 2026 NDoH workbook lists Ozempic, Wegovy injection, Extensor and Semagard registrations and prices. ⁵ Sun's 15 July 2026 disclosure separately confirms SAHPRA approval of Semagard injection for type 2 diabetes. ⁶ The same workbook contains no Rybelsus or Wegovy oral/tablet record, so South African registration and SEP for those oral products were not publicly verified. ⁵

FDA Orange Book data current 18 August 2026 show no approved semaglutide ANDA in the US, notwithstanding expiry of an older core patent in March 2026; listed product, formulation, use and device patents extend into 2038–2041 for selected products, and

product/indication exclusivities extend into 2027–2029. ⁹ FDA resolved the US semaglutide injection shortage in February 2025, while EMA marks its Ozempic shortage resolved as of an update on 13 January 2026; a current South African national status was not publicly verifiable. ⁵⁰ ⁵¹

Analysis and procurement implication

Analysis. Semaglutide is the best immediate category for a controlled competitive procurement. For diabetes pens, George should invite Novo and Sun/Ranbaxy to quote below SEP on exactly matched concentration, fill volume, deliverable doses, device and approved indication. Extensior is a pricing response within the Novo group, so true supplier diversification comes from Semagard, not merely from adding another Novo brand.

The tender or private RFP should not combine Wegovy obesity supply with diabetes products. Oral and injectable semaglutide must also remain in separate commercial models because formulation, pack duration, absorption technology and approved use differ. ⁷ Current certificates, approved professional information, batch-release records, cold-chain qualification and physical stock declarations should precede any award. ⁵ ³⁰

4.3 Insulin analogues: competitive molecules constrained by device ecosystems

Measured facts

South Africa's 5 August 2026 price file identifies registered/price-listed Lantus, Basaglar, Endulin Select, Tresiba, NovoRapid, Fiasp and Humalog presentations, but NDoH cautions that discontinued registered products may remain in the database. ⁵ ³⁰ Competition is strongest in glargine U-100, while degludec remains concentrated around Novo Nordisk's Tresiba in the reviewed markets. ³⁸ ³⁹

UK public price data show broadly matched follow-on list discounts of 13.7% for Semglee versus Lantus pens, 15.0% for Admelog versus Humalog vials and 15.0% for Trurapi versus NovoRapid vials, although the Drug Tariff can remain aligned with the reference product. ¹² ⁵³ EMA's 2025 communication records discontinuation of selected Fiasp, NovoRapid and Tresiba presentations and calls for supervised switching, showing that device/presentation cessation can disrupt supply even where a molecule remains marketed. ⁵⁴

Analysis and procurement implication

Analysis. George should make standard U-100 glargine its first insulin RFI, but only after confirming the exact Endulin Select presentation and current legal/manufacturing rights. Lots should be separated by molecule, concentration and device: glargine U-100 vial/cartridge/pen; prandial lispro and aspart with device sub-lots; and degludec U-100/U-

200 as a differentiated category. Toujeo U-300, Fiasp, concentrated lispro and unlike devices should not be forced into a lowest-price-per-mL auction.

The commercial award should include compatible needles, transition training, pharmacovigilance, minimum shelf life, 2–8°C lane validation, no-freeze controls, safety stock, allocation transparency and a 12–18 month discontinuation notice. The Biocon platform demonstrates why a competitive insulin supplier must integrate drug substance, sterile product, cartridge/vial and device capabilities; an API quote alone is not a substitutable medicine offer. ¹⁴

4.4 Dulaglutide: declining originator franchise, high biologic barriers

Measured facts

The South African NDoH database lists one Trulicity 1.5 mg/0.5 mL four-unit presentation, registration 51/21.13/0344 and NAPPI 3001765001, at ZAR 1,223.83 SEP including VAT, effective 24 February 2026. ⁵ No approved South African follow-on or public tender price was verified in the reviewed evidence.

Lilly reported worldwide Trulicity revenue of US\$7.132 billion in 2023, US\$5.254 billion in 2024 and US\$4.276 billion in 2025, a 40.0% decline over two years. ¹⁶ Boan Biotech's Boyouping received China NMPA approval on 8 August 2025, but FDA's biosimilar list and EMA's Trulicity record did not show a dulaglutide follow-on in their reviewed markets. ¹⁵ ⁵⁶ EMA recorded an all-strength Trulicity shortage beginning in 2023 and marked it resolved on 17 February 2026; Australia had used temporary authority for US-labelled pens, later lapsed in January 2026. ⁵⁷ ⁵⁸

Analysis and procurement implication

Analysis. George should treat Trulicity as a continuity contract with medium-term option value. Revenue decline, looming core-protection dates and the China biosimilar support future price pressure, but they do not create a South African second source today. ¹⁵ ¹⁶ Near-term negotiations should use the local SEP and franchise maturity to seek volume bands, expiry protection, service credits, allocation, and transparent rebate structures rather than citing US WAC.

A future development or licensing thesis requires a partner with CHO-cell biologics, glycosylation/comparability analytics, sterile fill-finish, integrated autoinjector, immunogenicity evidence, pharmacovigilance and cold-chain capability. Territorial rights and product-specific SAHPRA approval are prerequisites; China approval is not authority to market in South Africa. ¹⁵ ¹⁹

4.5 Octreotide and lanreotide: mature IR competition, complex depot economics

Measured facts

Short-acting octreotide is a conventional sterile solution with multiple regulated suppliers, whereas octreotide LAR is a refrigerated microsphere suspension and lanreotide is a prolonged-release semisolid gel in a prefilled syringe.^{22 24} South African SAHPRA evidence identifies Sandostatin IR registrations, while the Medicine Price Registry reports Sandostatin LAR, Teva octreotide depot and Somatuline registrations; current certificates should be obtained because the interactive SAHPRA register was not publicly extractable for every SKU.^{25 40}

National bid HP06-2024SVP requested 34,110 units of octreotide 0.05 mg/1 mL and 110,710 units of 0.1 mg/1 mL, but no valid public award price was verified.³¹ June 2026 NEMLC recommendations approved short-acting octreotide for persistent neonatal hyperinsulinism/hypoglycaemia and recorded somatostatin analogues for neuroendocrine tumours as not approved in the tertiary/quaternary EML decision history.⁵⁹ SAHPRA documented a Class II Type C recall initiated 27 June 2025 for specified Somatuline Autogel 90/120 mg batches due to possible needle detachment.⁴²

Analysis and procurement implication

Analysis. George should run separate RFIs for short-acting solution and depot products. Short-acting octreotide can be competed on exact concentration, preservative, pack, delivered price, remaining shelf life and allocation. Depot procurement should model a 28-day total service cost within a clinically valid regimen and include administration, training, product preparation, replacement doses, cold chain, homecare where applicable and wastage.

For octreotide LAR, a compliant Teva/Pharmathen route is the most relevant challenger to Novartis, subject to current South African registration and pack confirmation.^{23 41} For lanreotide, George should seek non-exclusive channel options and require evidence that the 2025 recall is closed, affected batches are segregated, corrective actions are effective, and replacement stock/service levels are documented.⁴² Bachem and CordenPharma are strategic capability signals, not finished-dose suppliers unless incorporated into an approved dossier and licensed supply chain.^{45 46}

5. South African availability and procurement implications

Category	Publicly evidenced South African position at 19 Aug 2026	Public tender/shortage visibility	Geographical impact
Tirzepatide	Mounjaro vials and KwikPens price-listed under Eli Lilly (SA); Aspen is local commercial partner. 1 34	No public Mounjaro award price or current SAHPRA shortage notice verified. 1	Single concentration; strong ceiling
Semaglutide	Ozempic, Wegovy injection, Extensior and Semagard price-listed; Sun confirms Semagard approval. 5 6	No public award price; current national shortage status not publicly verified. 5 50	Core product; strong regional
Insulin analogues	Multiple analogue products and applicants in the 5 Aug price file. 5	No current analogue award price or product-specific shortage bulletin verified; discontinued products can remain in database. 13 30	RFI stock; lot 1; more.
Dulaglutide	One Trulicity four-pen presentation price-listed as originator. 5	No public award price or current South African shortage notice verified.	Negative economic; retail; bio:
Octreotide/lanreotide	Sandostatin IR and depot/lanreotide registrations supported by mixed SAHPRA/MPR evidence; Acino named in Somatuline recall. 25 40 42	IR tender quantities published, but no valid award price; no current nationwide shortage verified. 31	Core product; recall

6. Supply-risk heat map

Scale: 1 = low relative risk; 5 = very high relative risk. Scores are analytical judgements derived from the cited supplier, manufacturing, device, shortage and regulatory evidence—not measured probabilities.

Category	Supplier concentration	Manufacturing / CMC complexity	Device / switching burden	Cold-chain logistics
Tirzepatide	5	4	4	4

Semaglutide	3	4	4	4
Insulin analogues	3	5	5	4
Dulaglutide	5	5	5	4
Short-acting octreotide	2	3	2	3
Octreotide LAR	4	5	5	5

Lanreotide depot	4	5	5	4
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7. Negotiation levers

Lever	Evidence base	Recommended use
Same-pack SEP competition	Semagard is 19.25% below Ozempic and 5.00% below Extensor at matched 1.5 mL; glargine U-100 records span ZAR 582.90–981.51 per nominal 1,500-unit pack. 5 6	Seek below-SEP offers with price held for a defined volume band; do not award until device, indication and availability match.
Mature-franchise pressure	Trulicity revenue declined 40.0% from 2023 to 2025, while China has an approved biosimilar. 15 16	Ask Lilly for retention pricing, expiry protection and volume rebates, without overstating current local competition.
Presentation economics	Four Mounjaro 2.5 mg vials cost ZAR 3,559.25 versus ZAR 3,605.59 for one four-dose KwikPen at SEP. 1	Negotiate bundled needles/syringes, training and dispensing support; award on total delivered cost and continuity.
Follow-on reference prices	UK follow-on list discounts are 10% for Olatuton, 20% for ADVANZ lanreotide and 13.7% for Semglee glargine. 12 27 28	Use as evidence that regulated follow-ons can support discounts, not as a South African target price or net-price proxy.
Service-level monetisation	Depot preparation complexity and the Somatuline needle recall create measurable operational risk. 23 42	Price replacement-dose guarantees, complaint turnaround, training, recall execution and local safety stock explicitly.
Allocation and continuity	Prior incretin shortages and insulin presentation discontinuations show that	Require fill-rate KPIs, allocations by SKU/strength, shortage notification,

	national adequacy does not eliminate local or presentation-level disruption. 48 50 54 57	substitute-transition plan and service credits.
Open-book price architecture	SEP includes manufacturer/logistics/VAT components, while WAC, tariffs and DPMQ have different rebate and reimbursement mechanics. 5 18 29	Request delivered VAT-inclusive pack price plus separately stated rebates, freight, consumables, training and wastage assumptions.
Portfolio bundling with safeguards	Lilly, Novo and Sanofi span multiple scoped categories. 1 5 36 39	Consider portfolio rebates only if they do not foreclose independent entrants, obscure SKU economics or increase single-supplier concentration.

8. Recommended portfolio and procurement stance

Time horizon	Category stance	Recommended action
0–6 months	Semaglutide diabetes and glargine U-100	Launch registration-and-capability RFIs, then competitive exact-presentation RFPs. Use SEP spreads as ceiling anchors and require live-stock evidence. 5 6 30
0–6 months	Tirzepatide and dulaglutide	Negotiate directly with registered Lilly/local channels for continuity, allocation, total-cost services and transparent rebates; do not claim a current second source. 1 5 34
0–9 months	Short-acting octreotide	Confirm current SAHPRA certificates and public/private demand, then compete exact concentration/pack with shelf-life and delivered-price SLAs. 31 40
0–12 months	Insulin prandial and differentiated basal lots	Build molecule/concentration/device-specific lots; include needles, training and transition support.

		Keep degludec and concentrated/faster products separate. 12 39 54
0–12 months	Octreotide LAR and lanreotide	Run non-binding complex-depot RFIs to Novartis, Teva/local registration chain and Somatuline channel; verify pack, sites, Zone IV stability, recall closure and device service. 25 42
12–36 months	Dulaglutide biosimilar and tirzepatide generic watch	Monitor primary regulatory filings, territorial rights and SAHPRA pathways. Proceed only with an end-to-end, approvable finished-dose/device partner. 3 15 47
Strategic	Local value creation	Prioritise licensed secondary packaging, validated cold-chain distribution, patient/provider device support and, only through approved change control, sterile fill-finish or technology transfer. Do not procure unregistered API as a substitute for an authorised medicine. 4 14 46

Portfolio conclusion. George should use a **barbell strategy**: capture near-term price competition where independent regulated suppliers already exist, while purchasing resilience and option value in concentrated categories. The portfolio should not be managed as “five peptides” ; it is a set of distinct regulated systems with different molecular pathways, devices, indications, sites and switching burdens.

9. Due-diligence checklist

Workstream	Required evidence before commitment
Product identity	Exact brand, INN, strength, concentration, fill volume, deliverable doses, dosage form, device, pack, NAPPI, schedule, indication and approved professional information.

South African authority	Current SAHPRA registration certificate and status; named local MAH/applicant; licensed importer, wholesaler and distributor; any Section 21 status separately identified.
Entity and site resolution	Legal contracting entity; ultimate parent; MAH; API/drug-substance site; drug-product/fill-finish site; device assembler; packaging site; importer; batch releaser; pharmacovigilance owner.
Quality and GMP	Current GMP evidence for every approved site; recent inspection history; batch-release specifications; peptide/protein impurity strategy; sterility assurance; container-closure integrity; device release controls; change-control history.
Supply availability	Written physical-stock declaration; allocation policy; lead time; minimum order; fill-rate history; open backorders; safety stock; remaining shelf life; 12–18 month discontinuation notice where applicable.
Cold chain	Label-specific 2–8°C/no-freeze requirements; validated lanes; data loggers; excursion-disposition authority; last-mile controls; returns/quarantine process; Zone IV stability evidence where applicable.
Device and consumables	Needle/syringe compatibility; included versus separately purchased consumables; human-factors evidence; training materials; sharps disposal; device complaint rate; replacement-dose process; medication-error mitigation.
Commercial terms	Delivered VAT-inclusive price per exact pack; distinction between SEP, discount, rebate and service fee; dispensing assumptions; freight; insurance; wastage; returns; currency and escalation terms.
Continuity and remedies	SKU/strength allocations; service-level KPIs; shortage notification; recall traceability; substitute-transition and retraining support; expedited replacement stock; service credits and termination rights.

Safety and pharmacovigilance	Local qualified pharmacovigilance contact; adverse-event and device-complaint reporting; recall SOP; field safety corrective action history; reconciliation between MAH, distributor and provider.
Regulatory/IP strategy	Product-specific SAHPRA pathway; dossier ownership/access; territorial rights; freedom-to-operate opinion for molecule, formulation, process and device; stability/bridging plan; no reliance on foreign approval alone.
Audit and data rights	Audit access to relevant quality and logistics systems; batch and temperature records; subcontractor disclosure; material-change notification; data retention; anti-counterfeit serialization/traceability.

10. Limitations

This is a public-source snapshot at 19 August 2026. Prices can change after the preserved source dates, and confidential tender discounts, payer rebates, chargebacks, wholesaler terms, patient assistance and manufacturer net realised prices were not available. The analysis therefore does not estimate net margin or rank countries by affordability.

NDoH SEP and medicine-price records provide registration-adjacent evidence but may retain discontinued products and do not prove physical stock.³⁰ Current product-specific SAHPRA certificates, approved professional information, live register status and licensed-supply arrangements must be obtained directly from bidders and the regulator before contracting.

No current South African public award price was verified for most scoped categories. Bid specifications, pipeline records, SEP and foreign reimbursement prices are not substituted for an award. Decentralised or confidential purchases may exist but cannot be inferred from the public record.

Supplier roles can differ across MAH, drug-substance manufacturer, fill-finish site, device assembler, labeler, importer and distributor. The entity appendix resolves names only to the extent supported by reviewed sources; it is not a legal-entity, sanctions, beneficial-ownership or GMP audit.

Patent and exclusivity dates are management or regulatory-listing observations, not legal freedom-to-operate opinions. Approval in the US, EU, UK, China or Australia does not

authorise South African marketing, and generic/biosimilar terminology is jurisdiction- and product-specific.

Arithmetic normalisations are descriptive procurement aids, not medical dosing advice. No recommendation is made to switch a patient, molecule, concentration, device, brand or indication. Clinical and formulary decisions require appropriately authorised professionals.

Appendix A. Entity resolution for named suppliers

Common name	Resolved legal identity / relationship	Role distinction relevant to George
Eli Lilly	Eli Lilly and Company ; local applicant Eli Lilly (SA) (Pty) Ltd ; other jurisdictional affiliates include Lilly USA, LLC and Eli Lilly Nederland B.V. 1 8 19	Parent/product owner is not always the invoicing MAH or local supplier; contract all responsibilities explicitly.
Aspen	Aspen Pharmacare Holdings Limited and its operating entities. 34	Tirzepatide distribution/promotion partner, not verified independent API or finished-dose manufacturer.
Sandoz	Sandoz Group AG and market affiliates. 3	Prospective tirzepatide generic applicant and conventional octreotide supplier in other markets; not approved South African tirzepatide source.
Novo Nordisk	Novo Nordisk A/S ; US applicant Novo Nordisk Inc.; local applicant Novo Nordisk (Pty) Ltd . 5 35	Originator/API/formulation owner; Extensor remains within Novo control.
Sun/Ranbaxy	Sun Pharmaceutical Industries Limited ; local subsidiary/applicant Ranbaxy Pharmaceuticals (Pty) Ltd . 5 6	Semagard is the relevant independent South African finished-dose competitor.
Sanofi	Sanofi S.A. ; relevant entities include Sanofi-Aventis South Africa (Pty) Ltd and Sanofi Winthrop Industrie. 5 37	Originator in glargine and competitor in rapid analogues; verify product-specific MAH/site.

Biocon	Biocon Biologics Limited , subsidiary of Biocon Limited . 14	Integrated insulin platform; manufacturer/approval owner can differ from commercial label.
Viartis/Mylan	Viartis Inc. ; local applicant Mylan (Pty) Ltd. 5	Endulin Select local record; current manufacturing and rights require product-level verification.
Civica	Civica, Inc. / CivicaScript LLC . 60	US private-label channel, not a South African registered supplier.
Boan	Shandong Boan Biotechnology Co., Ltd. 15	China-approved dulaglutide biosimilar developer/manufacturer; no assumed South African authority.
Shanghai Pharmaceuticals/Shaphar	Shanghai Pharmaceuticals Holding Co., Ltd. / Shanghai Pharmaceutical Co., Ltd. 15	Mainland-China commercialisation partner for Boyouping, not evidence of independent manufacture or SA supply.
Novartis	Novartis AG ; local registration holder Novartis South Africa (Pty) Ltd. 40	Sandostatin originator/MAH.
Teva	Teva Pharmaceutical Industries Ltd. and market affiliates. 41	Octreotide LAR follow-on marketer; Pharmathen is named finished-dose manufacturer for the US product.
Pharmathen	Pharmathen International S.A. 23	Private manufacturer supporting Teva's depot; not a direct route around registration.
Ipsen	Ipsen S.A. / Ipsen Pharma Biotech SAS. 24	Somatuline originator and device/formulation owner.
Acino South Africa	Acino Pharma (Pty) Ltd. 42	Local Somatuline supply entity named in SAHPRA recall evidence.
Cipla/InvaGen	Cipla Limited; Cipla USA Inc.; InvaGen Pharmaceuticals Inc.	US lanreotide follow-on group; applicant/labeler/parent

	43	names differ.
ADVANZ	ADVANZ PHARMA Corp. Limited / ADVANZ PHARMA Limited. 44	European lanreotide follow-on marketer; South African status not assumed.
Bachem	Bachem Holding AG. 45	Verified octreotide API capability; not a finished-dose registration.
CordenPharma	Corden Pharma International GmbH / CordenPharma group. 46	Peptide/API and sterile injectable CDMO capability; no approved scoped product supply inferred.

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